SAMPLE ANSWER · SPINAL CORD INJURY · CASE 5

# Sample 7-minute ISBAR: Robin

Robin Taylor · 52-year-old non-binary person · Inpatient rehabilitation unit, regional public hospital · about 995 spoken words (≈ 7.1 min at 140 words per minute)

## I Introduction

*59 words*

"Hi, I'm the occupational therapy student who has been doing car transfer practice with Robin Taylor. Robin is 52, uses they/them pronouns, and has T12 AIS C incomplete paraplegia after a fall from a roof five weeks ago. I need urgent help because I'm worried Robin may have a clot in their leg that has travelled to their lungs."

## S Situation

*119 words*

"During the first transfer, I noticed Robin's left calf is clearly bigger than the right, red and warm. Robin says it has felt tight since yesterday. In the car seat they had a sharp pain on the right side of the chest when breathing in deeply. They have felt breathless since this morning, even at rest, and they're breathing faster than usual. Their smartwatch shows a heart rate of 118, when they say they're normally in the seventies.

"Robin thinks it's nerves. Their first weekend home leave is due to start at 2 pm, and Mel has just arrived and is loading the wheelchair. I've stopped the session, and Robin is sitting in the car in the transfer bay."

## B Background

*148 words*

"Robin had spinal fusion surgery after the fall and moved to our unit in Townsville two weeks ago. Physio has documented slide-board transfers with standby assistance and cleared Robin for home leave. The medical team signed the leave form this morning, and nursing yesterday described Robin as independent with pressure care and in good spirits. OT to date has covered lower-body dressing, wheelchair skills and leave planning.

"Robin lives with their partner Mel and their daughter Ava, who turns 18 tomorrow, in a lowset house with three steps at the front. They're a self-employed carpenter and are worried about income and whether they have workers' compensation cover.

"Important gaps: there are no documented observations since 6 am, so I don't know Robin's blood pressure, oxygen levels or temperature. I don't know whether anyone has reviewed the leg, or whether Robin's daily blood-thinning injections have been given as charted."

## A Assessment

*252 words*

"A swollen, red, warm calf on one side suggests a possible deep vein thrombosis. Sharp chest pain on deep breathing, breathlessness at rest, faster breathing and a heart rate of 118 suggest that part of a clot may have travelled to the lungs, which would be a pulmonary embolism. That is a medical emergency and can be life-threatening.

"Robin is at high risk. People with spinal cord injury have a much higher risk of clots, especially in the first few months, because of reduced movement and changes in circulation. Robin also had recent spinal surgery. Their reduced sensation means the usual calf pain may not be there to warn them, which is probably why the leg didn't seem like a big deal to them.

"The reassuring information is from before today. Physio clearance, the signed leave form and yesterday's nursing notes don't tell us anything about these new signs. Robin's explanation that it's nerves is understandable, but anxiety doesn't cause one swollen, warm calf. Their rubbing the calf also worries me, because handling the leg could disturb a clot.

"Occupationally, any more transfers or a car trip home now could put Robin at serious risk, especially if they deteriorated away from medical help. The ramp, hand controls, the cushion question, work and the bank letter all matter to Robin and Mel, but none of them can be dealt with right now.

"As a student, I can't diagnose this or order investigations, but I think the signs are clear enough to escalate immediately."

## R Recommendation

*417 words*

"First, I've stopped the activity. I'd ask Robin to stay still in the car seat, sitting upright, and to stop rubbing the calf. I won't move or massage the leg or attempt another transfer. I'll stay with Robin and call for help immediately using the hospital's rapid response or MET call, because chest pain with breathlessness and a fast heart rate warrant urgent review. I'd follow the hospital's escalation process, and I wouldn't wait for a full set of observations first. I'd ask Mel to stay with us and stop loading the car.

"Second, I'd like your help to make sure the nurse in charge and the medical team are aware. Medical staff need to assess Robin urgently, check observations including oxygen levels, and decide on investigations and treatment. They also need to decide whether home leave can go ahead. Nursing can confirm whether the anticoagulant injections were given and monitor Robin closely. A safe way of getting Robin back to bed, such as a hoist, should be decided by the team rather than another transfer from me.

"Third, I'd document what I saw, the times and what I did, and complete an incident report if required.

"Once Robin is safe, I'd want the family's plans respected as much as possible. Ava's 18th is tomorrow and Robin has been counting down the hours. If Robin can't go home, I'd ask social work and nursing whether the family could celebrate on the ward. I'd also ask social work to help Robin check whether they have any workers' compensation or income protection cover, as WorkCover Queensland may not cover them as a self-employed person. Social work could also look into Centrelink and NDIS access for longer-term supports.

"For OT, once the medical team clears Robin for activity, my principal short-term assessment would be observed performance of the car transfer and getting into the house, to make sure leave is safe when it does happen. I'd answer the cushion question then, using Robin's pressure-care plan, so it's set up correctly for the car and at home.

"Longer term, I'd use the COPM, because Robin's priorities include family, income and returning to carpentry in some form. That would guide planning for the ramp, hand-control driving assessment through an OT driver assessor, and vocational rehab. Austroads *Assessing Fitness to Drive* would guide the driving question.

"My immediate priority is to keep Robin still, get emergency medical review now, and make sure they don't leave for home until the team has cleared them."

## Coaching notes *not spoken*

### The immediate priority

A swollen, warm, red calf plus pleuritic chest pain, breathlessness and a heart rate of 118 point to possible DVT with pulmonary embolism. Stop the activity, keep Robin still, don't touch the leg, and call a rapid response or MET call now.

### Distractors in this case

- **Physio clearance and the signed leave form** — these were done before the new signs appeared.
- **"It's nerves"** — anxiety doesn't explain a unilateral swollen, warm calf.
- **Mel's ramp and hand-control questions** — valid but not urgent.
- **Work, income, WorkCover, the bank letter** — real stressors; refer to social work later.
- **The cushion question** — answer later.
- **Ava's 18th and the 2 pm deadline** — emotional pressure, not a reason to proceed.

### Missing information to clarify

- Current observations, including oxygen saturation and blood pressure
- Whether anticoagulant doses have been given or missed
- Whether the leg has been reviewed
- Any past history of clots

### Useful references

- Austroads, *Assessing Fitness to Drive* (for the later driving question)
- Queensland Health, Queensland Adult Deterioration Detection System (Q-ADDS)

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